# Department of Health and Human Services

## Part 1. Overview Information

**Participating Organization(s)**

National Institutes of Health (NIH (http://www.nih.gov))

**Components of Participating Organizations**

National Institute of Diabetes and Digestive and Kidney Diseases (NIDDK (https://www.niddk.nih.gov/))

**Funding Opportunity Title**

### Advancing Research on the Application of Digital Health Technology to the Management of Type 2 Diabetes (R01- Clinical Trail Required)

**Activity Code**

R01 (//grants.nih.gov/grants/funding/ac_search_results.htm?text_curr=r01&Search.x=0&Search.y=0&Search_Type=Activity) Research Project Grant

**Announcement Type**

New

**Related Notices**

- **April 4, 2024** - Overview of Grant Application and Review Changes for Due Dates on or after January 25, 2025. See Notice NOT-OD-24-084 (https://grants.nih.gov/grants/guide/notice-files/NOT-OD-24-084.html).
- **August 31, 2022**- Implementation Changes for Genomic Data Sharing Plans Included with Applications Due on or after January 25, 2023. See Notice NOT-OD-22-198 (https://grants.nih.gov/grants/guide/notice-files/not-od-22-198.html).
- **August 5, 2022**- Implementation Details for the NIH Data Management and Sharing Policy. See Notice NOT-OD-22-189 (https://grants.nih.gov/grants/guide/notice-files/NOT-OD-22-189.html).

**Funding Opportunity Number (FON)**

RFA-DK-26-315

**Companion Funding Opportunity**

None

**Number of Applications**

See Part 2, Section III. 3. Additional Information on Eligibility.

**Assistance Listing Number(s)**

93.847

**Funding Opportunity Purpose**

This notice of funding opportunity (NOFO) invites applications designed to examine the efficacy of digital health technology approaches to the management of diabetes.

**Funding Opportunity Goal(s)**

Objectives
To promote extramural biomedical research that improves the understanding of disease management and leads to improved preventions, diagnosis, and treatment of diabetes, digestive, and kidney diseases. Programmatic areas within the National Institute of Diabetes and Digestive and Kidney Diseases include diabetes, digestive, endocrine, hematologic, liver, metabolic, nephrologic, nutrition, obesity, and urologic diseases.

## Key Dates

**Posted Date**

September 17, 2025

**Open Date (Earliest Submission Date)**

October 07, 2025

**Letter of Intent Due Date(s)**

Not applicable

| Application Due Dates | | | Review and Award Cycles | | |
|---|---|---|---|---|---|
| New | Renewal / Resubmission / Revision (as allowed) | AIDS - New/Renewal/Resubmission/Revision, as allowed | Scientific Merit Review | Advisory Council Review | Earliest Start Date |
| November 07, 2025 | Not Applicable | Not Applicable | March 2026 | May 2026 | July 2026 |
| October 06, 2026 | Not Applicable | Not Applicable | March 2027 | May 2027 | July 2027 |

All applications are due by 5:00 PM local time of applicant organization.

Applicants are encouraged to apply early to allow adequate time to make any corrections to errors found in the application during the submission process by the due date.

**Expiration Date**

October 07, 2026

**Due Dates for E.O. 12372**

Not Applicable

### Required Application Instructions

It is critical that applicants follow the instructions in the Research (R) Instructions in the How to Apply - Application Guide (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400), except where instructed to do otherwise (in this NOFO or in a Notice from NIH Guide for Grants and Contracts (//grants.nih.gov/grants/guide/url_redirect.php?id=11164)).

Conformance to all requirements (both in the Application Guide and the NOFO) is required and strictly enforced. Applicants must read and follow all application instructions in the Application Guide as well as any program-specific instructions noted in Section IV. When the program-specific instructions deviate from those in the Application Guide, follow the program-specific instructions.

**Applications that do not comply with these instructions may be delayed or not accepted for review.**

There are several options available to submit your application through Grants.gov to NIH and Department of Health and Human Services partners. You **must** use one of these submission options to access the application forms for this opportunity.

1. Use the NIH ASSIST system to prepare, submit and track your application online.

   Apply Online Using ASSIST

2. Use an institutional system-to-system (S2S) solution to prepare and submit your application to Grants.gov and eRA Commons (https://public.era.nih.gov/commons/) to track your application. Check with your institutional officials regarding availability.

3. Use Grants.gov (https://grants.gov/search-grants?oppStatuses=closed|archived|posted|forecasted&fon=RFA-DK-26-315) Workspace to prepare and submit your application and eRA Commons (http://public.era.nih.gov/commons/) to track your application.

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## Part 2. Full Text of Announcement

# Section I. Notice of Funding Opportunity Description

**Background**

The prevalence of type 2 diabetes (T2D) has expanded at epidemic proportions, afflicting 11% of the U.S. population and projected to grow to 13.8% or 45 million Americans with diabetes by 2030. The capacity of primary care providers to manage this growing number of persons with diabetes (PWD) is increasingly overwhelmed. This mismatch translates to limited or nonexistent interaction of many PWD with the health care system (HCS), clinical inertia among overwhelmed providers, an increasing degree of patient frustration and sense of helplessness and burden (diabetes distress), poor diabetes control, increased diabetes-related morbidity and mortality, and ultimately, increased HCS burden. A recent analysis of NHANES data shows that only one half of patients with diabetes are currently meeting A1c targets, blood pressure control targets, or lipid goals, and only 1 in 5 PWD currently are attaining acceptability targets in each of these basic measures of acceptable diabetes care. A paradigm shift in diabetes management is needed.

The increasing availability of FDA-cleared digital health technology (DHT) for type 2 diabetes (T2D) management holds great promise for addressing current barriers to care. DHT has the capacity to improve access to the health care system, promote and enhance patient engagement and sense of empowerment, improve glycemia and other metabolic outcomes, reduce clinical inertial and provider dissatisfaction, and provide favorable economics at the HCS level. Despite the promise of DHT applications in diabetes and prediabetes management, a rigorous evidenced-based foundation, particularly for multimodality digital health interventions, is lacking.

For this NOFO, DHT includes but is not limited to continuous glucose monitoring (CGM), telemedicine with virtual coaching, connected digital devices (e.g., smartwatches, activity monitors, digital scales), mobile health (mHealth) applications for monitoring clinically relevant parameters such dietary intake, medication adherence, mood, behavior, and methods to facilitate bidirectional information exchange with the health care team, including directed text messaging and patient portals. The application of digital interventions utilizing peer support mechanisms to improve diabetes care also are applicable to this NOFO.

**Special areas of research interest**

**General principles:**

- Multimodality DHT interventions that for example, combine CGM concurrently with other digital interventions in a virtual remote continuous care intervention, though not required, are of particular interest to this NOFO.
- All investigators are expected to include meaningful engagement approaches with the relevant research population in designing and implementing the proposed trial. This may involve engagement with patients, family members or caregivers, community members, community-based organization, clinicians and other care providers, health care systems, or other relevant partners throughout the research process. Meaningful engagement must entail more than focus groups, surveys, or other activities where partners are only involved as participants or respondents. Engagement must be designed to ensure that the research is practical and aligns with partners' preferences and values. Meaningful collaboration will lead to optimal trial recruitment and conduct as well as potential for future dissemination and sustainability.
- Multiple digital interventions have applicability to management of both T1 and T2 diabetes and these DHT interventions are of interest to this NOFO.
- Partnering with industry providers of virtual diabetes clinic offerings is allowed but not required.
- Clinical trials funded in response to this NOFO are expected to provide efficacy data that will enable future large multicenter clinical effectiveness studies on the application of DHT to PWD.

**Examples of DHT interventions to be tested include, but are not limited to:**

- **Continuous glucose monitoring:**
  - Studies on the efficacy of CGM in T2D, including in PWD not taking insulin
  - Studies on the utility of CGM in persons with prediabetes, for example, interventions that prevent the progression of prediabetes to diabetes
  - Studies examining the role of CGM in monitoring and adapting dietary interventions
  - Studies on the dose and duration of CGM usage necessary to effect change in health activation and healthy behaviors
  - Studies to assess and promote the accessibility, adoption and sustainability of CGM interventions
  - Studies assessing and improving the integration of CGM into clinic workflow, particularly in primary care settings
- **Activity monitoring:**
  - Studies examining the role of digital monitoring of activity on improvement of glycemia and other metabolic parameters in PWD
- **Digital nutritional monitoring**
  - The use of mobile applications to facilitate measures of macronutrient content of meals in PWD, and the impact of these interventions on diabetes-specific outcomes.
- **Telemedicine, directed text messaging, patient portals, and peer-support**
  - Studies assessing the incorporation of virtual communication into a multimodality virtual diabetes clinic model
  - Studies examining the influence of digital methods of multi-touchpoint peer support on glycemic and non-glycemic diabetes-specific outcomes
- **Gaming and gamification**
  - Studies examining the effect of digital gamification on the uptake and sustained adoption of diabetes education to improve knowledge and promote self-care behaviors (dietary, activity, and medication adherence) in a manner that improves diabetes-specific outcomes
- **DHT adoption and sustainment**
  - Studies examining factors predicting and improving patient engagement, adoption, and sustained use and utility of DHT interventions in PWD, and phenotyping according to acceptance of different modalities of DHT (CGM, wearables, telehealth, etc.)
  - Studies examining the role of interventions to improve access of DHT for all PWD; to evaluate, measure, and improve digital literacy; and examine the influence of health beliefs and trust in the HCS on the successful utilization of all forms of DHT, including telemedicine
  - Studies examining the influence of DHT interventions on patient engagement, self-empowerment, diabetes distress, and quality of life using validated measures.
  - Studies including a specific aim to determine the cost-effectiveness of specific DHT interventions in PWD
  - Studies examining the effect of incentivizing PWD, providers and the health care system to incorporate DHT into diabetes management
  - Studies examining the role of digital health navigators, including AI-based systems, in improving the acceptability and sustainability digital health interventions in PWD

**Applications Non-Responsive to this NOFO**

Applications proposing the types of studies listed below will be considered non-responsive and will not be reviewed or considered for funding**.**

- Studies of automated insulin delivery systems including the artificial pancreas and hybrid closed loop technology
- Technologies (such as ketone monitoring) exclusively utilized in patients with T1D are not responsive to this NOFO.
- Projects developing new digital health technologies or algorithms are not permitted for this NOFO.
- Research projects utilizing DHT data to classify or further subtype diabetes or prediabetes are not permitted.
- Mechanistic studies, including mechanistic clinical trials are not responsive to this NOFO.

Investigators proposing NIH-defined clinical trials may refer to the Research Methods Resources (https://researchmethodsresources.nih.gov/) website for information about developing statistical methods and study designs.

See Section VIII. Other Information for award authorities and regulations.

# Section II. Award Information

**Funding Instrument**

Grant: A financial assistance mechanism providing money, property, or both to an eligible entity to carry out an approved project or activity.

**Application Types Allowed**

New

The OER Glossary (//grants.nih.gov/grants/guide/url_redirect.php?id=11116) and the How to Apply Application Guide provide details on these application types. Only those application types listed here are allowed for this NOFO.

**Clinical Trial?**

Required: Only accepting applications that propose clinical trial(s).

Need help determining whether you are doing a clinical trial? (https://grants.nih.gov/grants/guide/url_redirect.php?id=82370)

**Funds Available and Anticipated Number of Awards**

3 grants to be awarded in FY2026 and FY 2027. Direct costs and funds available will be 1.5M in 2026, 3.0M in FYs 2027-2030, and 1.5M in FY2031.

**Award Budget**

Application budgets are not limited but need to reflect the actual needs of the proposed project.

**Award Project Period**

The scope of the proposed project should determine the project period. The maximal project period is 5 years.

NIH grants policies as described in the NIH Grants Policy Statement (//grants.nih.gov/grants/guide/url_redirect.php?id=11120) will apply to the applications submitted and awards made from this NOFO.

# Section III. Eligibility Information

## 1. Eligible Applicants

### Eligible Organizations

Higher Education Institutions - Includes all types

- Public/State Controlled Institutions of Higher Education
- Private Institutions of Higher Education

Nonprofits Other Than Institutions of Higher Education

- Nonprofits with 501(c)(3) IRS Status (Other than Institutions of Higher Education)
- Nonprofits without 501(c)(3) IRS Status (Other than Institutions of Higher Education)

For-Profit Organizations

- Small Businesses
- For-Profit Organizations (Other than Small Businesses)

Local Governments

- State Governments
- County Governments
- City or Township Governments
- Special District Governments
- Indian/Native American Tribal Governments (Federally Recognized)
- Indian/Native American Tribal Governments (Other than Federally Recognized).

Federal Governments

- Eligible Agencies of the Federal Government
- U.S. Territory or Possession

Other

- Independent School Districts
- Public Housing Authorities/Indian Housing Authorities
- Native American Tribal Organizations (other than Federally recognized tribal governments)
- Faith-based or Community-based Organizations
- Regional Organizations

### Foreign Organizations

Non-domestic (non-U.S.) Entities (Foreign Organizations) **are not** eligible to apply.

Non-domestic (non-U.S.) components of U.S. Organizations **are not** eligible to apply.

Foreign components, as defined in the NIH Grants Policy Statement (//grants.nih.gov/grants/guide/url_redirect.php?id=11118), **are not** allowed.

### Required Registrations

**Applicant Organizations**

Applicant organizations must complete and maintain the following registrations as described in the How to Apply- Application Guide to be eligible to apply for or receive an award. All registrations must be completed prior to the application being submitted. Registration can take 6 weeks or more, so applicants should begin the registration process as soon as possible. Failure to complete registrations in advance of a due date is not a valid reason for a late submission, please reference the NIH Grants Policy Statement Section 2.3.9.2 Electronically Submitted Applications (//grants.nih.gov/grants/guide/url_redirect.php?id=82423) for additional information.

- System for Award Management (SAM) – (https://grants.nih.gov/grants/guide/url_redirect.php?id=82390) Applicants must complete and maintain an active registration, **which requires renewal at least annually**. The renewal process may require as much time as the initial

registration. SAM registration includes the assignment of a Commercial and Government Entity (CAGE) Code for domestic organizations which have not already been assigned a CAGE Code.
    - NATO Commercial and Government Entity (NCAGE) Code (//grants.nih.gov/grants/guide/url_redirect.php?id=11176) – Foreign organizations must obtain an NCAGE code (in lieu of a CAGE code) in order to register in SAM.
    - Unique Entity Identifier (UEI) - A UEI is issued as part of the SAM.gov registration process. The same UEI must be used for all registrations, as well as on the grant application.
- eRA Commons (https://grants.nih.gov/grants/guide/url_redirect.php?id=11123) - Once the unique organization identifier is established, organizations can register with eRA Commons in tandem with completing their Grants.gov registrations; all registrations must be in place by time of submission. eRA Commons requires organizations to identify at least one Signing Official (SO) and at least one Program Director/Principal Investigator (PD/PI) account in order to submit an application.
- Grants.gov (//grants.nih.gov/grants/guide/url_redirect.php?id=82300) – Applicants must have an active SAM registration in order to complete the Grants.gov registration.

**Program Directors/Principal Investigators (PD(s)/PI(s))**

All PD(s)/PI(s) must have an eRA Commons account. PD(s)/PI(s) should work with their organizational officials to either create a new account or to affiliate their existing account with the applicant organization in eRA Commons. If the PD/PI is also the organizational Signing Official, they must have two distinct eRA Commons accounts, one for each role. Obtaining an eRA Commons account can take up to 2 weeks.

### Eligible Individuals (Program Director/Principal Investigator)

Any individual(s) with the skills, knowledge, and resources necessary to carry out the proposed research as the Program Director(s)/Principal Investigator(s) (PD(s)/PI(s)) is invited to work with their organization to develop an application for support.

For institutions/organizations proposing multiple PDs/PIs, visit the Multiple Program Director/Principal Investigator Policy and submission details in the Senior/Key Person Profile (Expanded) Component of the How to Apply-Application Guide.

### 2. Cost Sharing

This NOFO does not require cost sharing as defined in the NIH Grants Policy Statement Section 1.2 Definition of Terms (//grants.nih.gov/grants/guide/url_redirect.php?id=11126).

### 3. Additional Information on Eligibility

Number of Applications

Applicant organizations may submit more than one application, provided that each application is scientifically distinct.

The NIH will not accept duplicate or highly overlapping applications under review at the same time, per NIH Grants Policy Statement Section 2.3.7.4 Submission of Resubmission Application (//grants.nih.gov/grants/guide/url_redirect.php?id=82415). This means that the NIH will not accept:

- A new (A0) application that is submitted before issuance of the summary statement from the review of an overlapping new (A0) or resubmission (A1) application.
- A resubmission (A1) application that is submitted before issuance of the summary statement from the review of the previous new (A0) application.
- An application that has substantial overlap with another application pending appeal of initial peer review (see NIH Grants Policy Statement 2.3.9.4 Similar, Essentially Identical, or Identical Applications (//grants.nih.gov/grants/guide/url_redirect.php?id=82423)).

## Section IV. Application and Submission Information

### 1. Requesting an Application Package

The application forms package specific to this opportunity must be accessed through ASSIST, Grants.gov Workspace or an institutional system-to-system solution. Links to apply using ASSIST or Grants.gov Workspace are available in Part 1 of this NOFO. See your administrative office for instructions if you plan to use an institutional system-to-system solution.

### 2. Content and Form of Application Submission

It is critical that applicants follow the instructions in the Research (R) Instructions in the How to Apply - Application Guide (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400) except where instructed in this notice of funding opportunity to do otherwise. Conformance to the requirements in the Application Guide is required and strictly enforced. Applications that are out of compliance with these instructions may be delayed or not accepted for review.

### Page Limitations

All page limitations described in the How to Apply- Application Guide (https://grants.nih.gov/grants/how-to-apply-application-guide.html) and the Table of Page Limits (https://grants.nih.gov/grants/guide/url_redirect.php?id=61134) must be followed.

### Instructions for Application Submission

The following section supplements the instructions found in the How to Apply- Application Guide (https://grants.nih.gov/grants/how-to-apply-application-guide.html) and should be used for preparing an application to this NOFO.

### SF424(R&R) Cover

All instructions in the How to Apply - Application Guide (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400) must be followed.

### SF424(R&R) Project/Performance Site Locations

All instructions in the How to Apply- Application Guide must be followed.

### SF424(R&R) Other Project Information

All instructions in the How to Apply- Application Guide must be followed.

### SF424(R&R) Senior/Key Person Profile

All instructions in the How to Apply- Application Guide must be followed.

### R&R or Modular Budget

All instructions in the How to Apply- Application Guide must be followed.

### R&R Subaward Budget

All instructions in the How to Apply-Application Guide must be followed.

### PHS 398 Cover Page Supplement

All instructions in the How to Apply- Application Guide must be followed.

### PHS 398 Research Plan

All instructions in the How to Apply- Application Guide must be followed, with the following additional instructions:

**Resource Sharing Plan**: Individuals are required to comply with the instructions for the Resource Sharing Plans as provided in the How to Apply- Application Guide.

**Other Plan(s):**

All instructions in the How to Apply-Application Guide must be followed, with the following additional instructions:

- All applicants planning research (funded or conducted in whole or in part by NIH) that results in the generation of scientific data are required to comply with the instructions for the Data Management and Sharing Plan. All applications, regardless of the amount of direct costs requested for any one year, must address a Data Management and Sharing Plan.

**Appendix:** Only limited Appendix materials are allowed. Follow all instructions for the Appendix as described in the How to Apply- Application Guide.

- No publications or other material, with the exception of blank questionnaires or blank surveys, may be included in the Appendix.

### PHS Human Subjects and Clinical Trials Information

When involving human subjects research, clinical research, and/or NIH-defined clinical trials (and when applicable, clinical trials research experience) follow all instructions for the PHS Human Subjects and Clinical Trials Information form in the How to Apply- Application Guide, with the following additional instructions:

If you answered "Yes" to the question "Are Human Subjects Involved?" on the R&R Other Project Information form, you must include at least one human subjects study record using the **Study Record: PHS Human Subjects and Clinical Trials Information** form or **Delayed Onset Study** record.

**Study Record: PHS Human Subjects and Clinical Trials Information**

All instructions in the How to Apply- Application Guide must be followed.

**Delayed Onset Study**

Note: Delayed onset (https://grants.nih.gov/grants/glossary.htm#DelayedOnsetStudy) does NOT apply to a study that can be described but will not start immediately (i.e., delayed start). All instructions in the How to Apply- Application Guide must be followed.

### PHS Assignment Request Form

All instructions in the How to Apply- Application Guide must be followed.

### 3. Unique Entity Identifier and System for Award Management (SAM)

See Part 2. Section III.1 for information regarding the requirement for obtaining a unique entity identifier and for completing and maintaining active registrations in System for Award Management (SAM), NATO Commercial and Government Entity (NCAGE) Code (if applicable), eRA Commons, and Grants.gov

### 4. Submission Dates and Times

Part I. contains information about Key Dates and times. Applicants are encouraged to submit applications before the due date to ensure they have time to make any application corrections that might be necessary for successful submission. When a submission date falls on a weekend or Federal holiday (https://grants.nih.gov/grants/guide/url_redirect.php?id=82380), the application deadline is automatically extended to the next business day.

Organizations must submit applications to Grants.gov (//grants.nih.gov/grants/guide/url_redirect.php?id=11128) (the online portal to find and apply for grants across all Federal agencies). Applicants must then complete the submission process by tracking the status of the application in the eRA Commons (//grants.nih.gov/grants/guide/url_redirect.php?id=11123), NIH's electronic system for grants administration. NIH and Grants.gov systems check the application against many of the application instructions upon submission. Errors must be corrected and a changed/corrected application must be submitted to Grants.gov on or before the application due date and time. If a Changed/Corrected application is submitted after the deadline, the application will be considered late. Applications that miss the due date and time are subjected to

the NIH Grants Policy Statement Section 2.3.9.2 Electronically Submitted Applications (//grants.nih.gov/grants/guide/url_redirect.php?id=82423).

**Applicants are responsible for viewing their application before the due date in the eRA Commons to ensure accurate and successful submission.**

Information on the submission process and a definition of on-time submission are provided in the How to Apply-Application Guide.

### 5. Intergovernmental Review (E.O. 12372)

This initiative is not subject to intergovernmental review. (https://grants.nih.gov/grants/policy/nihgps/html5/section_10/10.10.1_executive_orders.htm)

### 6. Funding Restrictions

All NIH awards are subject to the terms and conditions, cost principles, and other considerations described in the NIH Grants Policy Statement (//grants.nih.gov/grants/guide/url_redirect.php?id=11120).

Pre-award costs are allowable only as described in the NIH Grants Policy Statement Section 7.9.1 Selected Items of Cost. (//grants.nih.gov/grants/guide/url_redirect.php?id=11143)

### 7. Other Submission Requirements and Information

Applications must be submitted electronically following the instructions described in the How to Apply Application Guide. Paper applications will not be accepted.

Applicants must complete all required registrations before the application due date. Section III. Eligibility Information contains information about registration.

For assistance with your electronic application or for more information on the electronic submission process, visit How to Apply – Application Guide (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400). If you encounter a system issue beyond your control that threatens your ability to complete the submission process on-time, you must follow the Dealing with System Issues (https://grants.nih.gov/grants/how-to-apply-application-guide/due-dates-and-submission-policies/dealing-with-system-issues.htm) guidance. For assistance with application submission, contact the Application Submission Contacts in Section VII.

**Important reminders:**

All PD(s)/PI(s) must include their eRA Commons ID in the Credential field of the Senior/Key Person Profile form**.** Failure to register in the Commons and to include a valid PD/PI Commons ID in the credential field will prevent the successful submission of an electronic application to NIH. See Section III of this NOFO for information on registration requirements.

The applicant organization must ensure that the unique entity identifier provided on the application is the same identifier used in the organization's profile in the eRA Commons and for the System for Award Management. Additional information may be found in the How to Apply Application Guide.

See more tips (//grants.nih.gov/grants/guide/url_redirect.php?id=11146) for avoiding common errors.

Upon receipt, applications will be evaluated for completeness and compliance with application instructions by the Center for Scientific Review and responsiveness by components of participating organizations, NIH. Applications that are incomplete, non-compliant and/or nonresponsive will not be reviewed.

### Requests of $500,000 or more for direct costs in any year

Applicants requesting $500,000 or more in direct costs in any year (excluding consortium F&A) must contact a Scientific/ Research Contact at least 6 weeks before submitting the application and follow the Policy on the Acceptance for Review of Unsolicited Applications that Request $500,000 or More in Direct Costs as described in the How to Apply-Application Guide.

### Mandatory Disclosure

Recipients or subrecipients must submit any information related to violations of federal criminal law involving fraud, bribery, or gratuity violations potentially affecting the federal award. See Mandatory Disclosures, 2 CFR 200.113 (https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200/subpart-B/section-200.113) and NIH Grants Policy Statement Section 4.1.35 (https://grants.nih.gov/grants/policy/nihgps/html5/section_4/4.1.35_mandatory_disclosures.htm).

Send written disclosures to the NIH Chief Grants Management Officer listed on the Notice of Award for the IC that funded the award and to the HHS Office of Inspector Grant Self Disclosure Program (https://oig.hhs.gov/compliance/self-disclosure-info/hhs-oig-grant-self-disclosure-program/) at grantdisclosures@oig.hhs.gov (mailto:grantdisclosures@oig.hhs.gov).

### Post Submission Materials

Applicants are required to follow the instructions for post-submission materials, as described in the policy (//grants.nih.gov/grants/guide/url_redirect.php?id=82299)

Any instructions provided here are in addition to the instructions in the policy.

## Section V. Application Review Information

### 1. Criteria

Only the review criteria described below will be considered in the review process. Applications submitted to the NIH in support of the NIH mission (//grants.nih.gov/grants/guide/url_redirect.php?id=11149) are evaluated for scientific and technical merit through the NIH peer review

system.
For this particular announcement, note the following:

## Overall Impact

Reviewers will provide an overall impact score to reflect their assessment of the likelihood for the project to exert a sustained, powerful influence on the research field(s) involved, in consideration of the following scored review criteria and additional review criteria (as applicable for the project proposed). An application does not need to be strong in all categories to be judged likely to have a major scientific impact.

## Scored Review Criteria

Reviewers will consider Factors 1, 2 and 3 in the determination of scientific merit, and in providing an overall impact score. In addition, Factors 1 and 2 will each receive a separate factor score.

### Factor 1. Importance of the Research (Significance and Innovation)

***Significance***

- Evaluate the importance of the proposed research in the context of current scientific challenges and opportunities, either for advancing knowledge within the field, or more broadly. Assess whether the application addresses an important gap in knowledge in the field, would solve a critical problem, or create a valuable conceptual or technical advance.
- Evaluate the rationale for undertaking the study, the rigor of the scientific background for the work (e.g., prior literature and/or preliminary data) and whether the scientific background justifies the proposed study.

***Innovation***

- Evaluate the extent to which innovation influences the importance of undertaking the proposed research. Note that while technical or conceptual innovation can influence the importance of the proposed research, a project that is not applying novel concepts or approaches may be of critical importance for the field.
- Evaluate whether the proposed work applies novel concepts, methods or technologies or uses existing concepts, methods, technologies in novel ways, to enhance the overall impact of the project.

### Factor 2. Rigor and Feasibility (Approach)

***Approach***

- Evaluate the scientific quality of the proposed work. Evaluate the likelihood that compelling, reproducible findings will result (rigor) and assess whether the proposed studies can be done well and within the timeframes proposed (feasibility).

***Rigor:***

- Evaluate the potential to produce unbiased, reproducible, robust data.
- Evaluate the rigor of experimental design and whether appropriate controls are in place.
- Evaluate whether the sample size is sufficient and well-justified.
- Assess the quality of the plans for analysis, interpretation, and reporting of results.
- Evaluate whether the investigators presented adequate plans to address relevant biological variables, such as sex or age, in the design, analysis, and reporting.
- For applications involving human subjects or vertebrate animals, also evaluate:
    - the rigor of the intervention or study manipulation (if applicable to the study design).
    - whether outcome variables are justified.
    - whether the results will be generalizable or, in the case of a rare disease/special group, relevant to the particular subgroup.
    - whether the study population appropriately models the target population.
- For applications involving human subjects, including clinical trials, assess the adequacy of inclusion plans as appropriate for the scientific goals of the research. Considerations of appropriateness may include disease/condition/behavior incidence, prevalence, or population burden, population representation, and/or current state of the science.

***Feasibility:***

- Evaluate whether the proposed approach is sound and achievable, including plans to address problems or new challenges that emerge in the work. For proposed studies in which feasibility may be less certain, evaluate whether the uncertainty is balanced by the potential for major advances.
- For applications involving human subjects, including clinical trials, evaluate the adequacy and feasibility of the plan to recruit and retain a study population that appropriately models the target population. Additionally, evaluate the likelihood of successfully achieving the proposed enrollment based on age, race, ethnicity, and sex.
- For clinical trial applications, evaluate whether the study timeline and milestones are feasible.

### Factor 3. Expertise and Resources (Investigator(s) and Environment)

***Investigator(s)***

Evaluate whether the investigator(s) have demonstrated background, training, and expertise, as appropriate for their career stage, to conduct the proposed work. For Multiple Principal Investigator (MPI) applications, assess the quality of the leadership plan to facilitate coordination and collaboration.

***Environment***

Evaluate whether the institutional resources are appropriate to ensure the successful execution of the proposed work.

## Additional Review Criteria

As applicable for the project proposed, reviewers will consider the following additional items while determining scientific and technical merit, but will not give criterion scores for these items, and should consider them in providing an overall impact score.

### Protections for Human Subjects

For research that involves human subjects but does not involve one of the categories of research that are exempt under 45 CFR Part 46, evaluate the justification for involvement of human subjects and the proposed protections from research risk relating to their participation according to the following five review criteria: 1) risk to subjects; 2) adequacy of protection against risks; 3) potential benefits to the subjects and others; 4) importance of the knowledge to be gained; and 5) data and safety monitoring for clinical trials.

For research that involves human subjects and meets the criteria for one or more of the categories of research that are exempt under 45 CFR Part 46, evaluate: 1) the justification for the exemption; 2) human subjects involvement and characteristics; and 3) sources of materials. For additional information on review of the Human Subjects section, please refer to the Guidelines for the Review of Human Subjects (https://grants.nih.gov/grants/guide/url_redirect.php?id=11175).

### Vertebrate Animals

When the proposed research includes Vertebrate Animals, evaluate the involvement of live vertebrate animals according to the following criteria: (1) description of proposed procedures involving animals, including species, strains, ages, sex, and total number to be used; (2) justifications for the use of animals versus alternative models and for the appropriateness of the species proposed; (3) interventions to minimize discomfort, distress, pain and injury; and (4) justification for euthanasia method if NOT consistent with the AVMA Guidelines for the Euthanasia of Animals. For additional information on review of the Vertebrate Animals section, please refer to the Worksheet for Review of the Vertebrate Animals Section (//grants.nih.gov/grants/guide/url_redirect.php?id=11150).

### Biohazards

When the proposed research includes Biohazards, evaluate whether specific materials or procedures that will be used are significantly hazardous to research personnel and/or the environment, and whether adequate protection is proposed.

### Resubmissions

As applicable, evaluate the full application as now presented.

### Renewals

As applicable, evaluate the progress made in the last funding period.

### Revisions

As applicable, evaluate the appropriateness of the proposed expansion of the scope of the project.

## Additional Review Considerations

As applicable for the project proposed, reviewers will consider each of the following items, but will not give scores for these items, and should not consider them in providing an overall impact score.

### Authentication of Key Biological and/or Chemical Resources

For projects involving key biological and/or chemical resources, evaluate the brief plans proposed for identifying and ensuring the validity of those resources.

### Budget and Period of Support

Evaluate whether the budget and the requested period of support are fully justified and reasonable in relation to the proposed research.

## 2. Review and Selection Process

Applications will be evaluated for scientific and technical merit by (an) appropriate Scientific Review Group(s), convened by CSR, in accordance with NIH peer review policies and practices (https://grants.nih.gov/policy-and-compliance/policy-topics/peer-review), using the stated review criteria. Assignment to a Scientific Review Group will be shown in the eRA Commons.

As part of the scientific peer review, all applications will receive a written critique.

Applications may undergo a selection process in which only those applications deemed to have the highest scientific and technical merit (generally the top half of applications under review) will be discussed and assigned an overall impact score.

Appeals (https://grants.nih.gov/grants/policy/nihgps/html5/section_2/2.4.2_appeals_of_initial_scientific_review.htm) of initial peer review will not be accepted for applications submitted in response to this NOFO.

Applications will be assigned to the appropriate NIH Institute or Center. Applications will compete for available funds with all other recommended applications submitted in response to this NOFO. Following initial peer review, recommended applications will receive a second level of review by the National Diabetes and Digestive and Kidney Diseases Advisory Council. The following will be considered in making funding decisions:

- Scientific and technical merit of the proposed project as determined by scientific peer review.
- Availability of funds.
- Relevance of the proposed project to program priorities.

If the application is under consideration for funding, NIH will request "just-in-time" information from the applicant as described in the NIH Grants Policy Statement Section 2.5.1. Just-in-Time Procedures (//grants.nih.gov/grants/guide/url_redirect.php?id=82418). This request is not a Notice of Award nor should it be construed to be an indicator of possible funding.

Prior to making an award, NIH reviews an applicant's federal award history in SAM.gov to ensure sound business practices. An applicant can review and comment on any information in the Responsibility/Qualification records available in SAM.gov. NIH will consider any comments by the applicant in the Responsibility/Qualification records in SAM.gov to ascertain the applicant's integrity, business ethics, and performance record of managing Federal awards per 2 CFR Part 200.206 "Federal awarding agency review of risk posed by applicants." This provision will apply to all NIH grants and cooperative agreements except fellowships.

### 3. Anticipated Announcement and Award Dates

After the peer review of the application is completed, the PD/PI will be able to access his or her Summary Statement (written critique) via the eRA Commons (//grants.nih.gov/grants/guide/url_redirect.php?id=11123). Refer to Part 1 for dates for peer review, advisory council review, and earliest start date.

Information regarding the disposition of applications is available in the NIH Grants Policy Statement Section 2.4.4 Disposition of Applications (//grants.nih.gov/grants/guide/url_redirect.php?id=82416).

## Section VI. Award Administration Information

### 1. Award Notices

A Notice of Award (NoA) is the official authorizing document notifying the applicant that an award has been made and that funds may be requested from the designated HHS payment system or office. The NoA is signed by the Grants Management Officer and emailed to the recipient's business official.

In accepting the award, the recipient agrees that any activities under the award are subject to all provisions currently in effect or implemented during the period of the award, other Department regulations and policies in effect at the time of the award, and applicable statutory provisions.

Recipients must comply with any funding restrictions described in Section IV.6. Funding Restrictions. Any pre-award costs incurred before receipt of the NoA are at the applicant's own risk. For more information on the Notice of Award, please refer to the NIH Grants Policy Statement Section 5. The Notice of Award (https://grants.nih.gov/grants/policy/nihgps/HTML5/section_5/5_the_notice_of_award.htm) and NIH Grants & Funding website, see Award Process. (https://grants.nih.gov/grants/pre-award-process.htm#award)

Individual awards are based on the application submitted to, and as approved by, the NIH and are subject to the IC-specific terms and conditions identified in the NoA.

ClinicalTrials.gov: If an award provides for one or more clinical trials. By law (Title VIII, Section 801 of Public Law 110-85), the "responsible party" must register and submit results information for certain "applicable clinical trials" on the ClinicalTrials.gov Protocol Registration and Results System Information Website (https://register.clinicaltrials.gov (https://register.clinicaltrials.gov/)). NIH expects registration and results reporting of all trials whether required under the law or not. For more information, see https://grants.nih.gov/policy/clinical-trials/reporting/index.htm (https://grants.nih.gov/policy/clinical-trials/reporting/index.htm)

Institutional Review Board or Independent Ethics Committee Approval: Recipient institutions must ensure that all protocols are reviewed by their IRB or IEC. To help ensure the safety of participants enrolled in NIH-funded studies, the recipient must provide NIH copies of documents related to all major changes in the status of ongoing protocols.

Data and Safety Monitoring Requirements: The NIH policy for data and safety monitoring requires oversight and monitoring of all NIH-conducted or -supported human biomedical and behavioral intervention studies (clinical trials) to ensure the safety of participants and the validity and integrity of the data. Further information concerning these requirements is found at http://grants.nih.gov/grants/policy/hs/data_safety.htm (//grants.nih.gov/grants/policy/hs/data_safety.htm) and in the application instructions (SF424 (R&R) and PHS 398).

Investigational New Drug or Investigational Device Exemption Requirements: Consistent with federal regulations, clinical research projects involving the use of investigational therapeutics, vaccines, or other medical interventions (including licensed products and devices for a purpose other than that for which they were licensed) in humans under a research protocol must be performed under a Food and Drug Administration (FDA) investigational new drug (IND) or investigational device exemption (IDE).

### 2. Administrative and National Policy Requirements

The following Federal wide and HHS-specific policy requirements apply to awards funded through NIH:

- The rules listed at 2 CFR Part 200 (https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200), Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards.
- All NIH grant and cooperative agreement awards include the NIH Grants Policy Statement (//grants.nih.gov/grants/guide/url_redirect.php?id=11120) as part of the terms and conditions in the Notice of Award (NoA). The NoA includes the requirements of this NOFO. For these terms of award, see the NIH Grants Policy Statement Part II: Terms and Conditions of NIH Grant Awards, Subpart A: General (//grants.nih.gov/grants/guide/url_redirect.php?id=11120) and Part II: Terms and Conditions of NIH Grant Awards, Subpart B: Terms and Conditions for Specific Types of Grants, Recipients, and Activities (//grants.nih.gov/grants/guide/url_redirect.php?id=11159).

All federal statutes and regulations relevant to federal financial assistance, including those highlighted in NIH Grants Policy Statement Section 4 Public Policy Requirements, Objectives and Other Appropriation Mandates. (https://grants.nih.gov/grants/policy/nihgps/HTML5/section_4/4_public_policy_requirements__objectives_and_other_appropriation_mandates.htm)

Recipients are responsible for ensuring that their activities comply with all applicable federal regulations. NIH may terminate awards under certain circumstances. See 2 CFR Part 200.340 Termination (https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200/subpart-D/subject-group-ECFR86b76dde0e1e9dc/section-200.340) and NIH Grants Policy Statement Section 8.5.2 Remedies for Noncompliance or

Enforcement Actions: Suspension, Termination, and Withholding of Support (https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.5.2_remedies_for_noncompliance_or_enforcement_actions-_suspension__termination__and_withholding_of_support.htm).

Pursuant to the Cybersecurity Act of 2015, Div. N, § 405, Pub. Law 114-113, 6 USC § 1533(d), the HHS Secretary has established a common set of voluntary, consensus-based, and industry-led guidelines, best practices, methodologies, procedures, and processes.

Successful recipients under this NOFO agree that:

When recipients, subrecipients, or third-party entities have:

1. ongoing and consistent access to HHS owned or operated information or operational technology systems; and
2. receive, maintain, transmit, store, access, exchange, process, or utilize personal identifiable information (PII) or personal health information (PHI) obtained from the awarding HHS agency for the purposes of executing the award.

Recipients shall develop plans and procedures, modeled after the NIST Cybersecurity framework (https://www.nist.gov/cyberframework), to protect HHS systems and data. Please refer to NIH Post-Award Monitoring and Reporting (https://grants.nih.gov/grants/post-award-monitoring-and-reporting.htm) for additional information.

### Cooperative Agreement Terms and Conditions of Award

Not Applicable

### 3. Data Management and Sharing

Consistent with the 2023 NIH Policy for Data Management and Sharing, when data management and sharing is applicable to the award, recipients will be required to adhere to the Data Management and Sharing requirements as outlined in the NIH Grants Policy Statement (https://grants.nih.gov/grants/policy/nihgps/HTML5/section_8/8.2.3_sharing_research_resources.htm#Data). Upon the approval of a Data Management and Sharing Plan, it is required for recipients to implement the plan as described.

### 4. Reporting

When multiple years are involved, recipients will be required to submit the Research Performance Progress Report (RPPR) (//grants.nih.gov/grants/rppr/index.htm) annually and financial statements as required in the NIH Grants Policy Statement Section 8.4.1 Reporting. (https://grants.nih.gov/grants/policy/nihgps/HTML5/section_8/8.4.1_reporting.htm) To learn more about post-award monitoring and reporting, see the NIH Grants & Funding website, see Post-Award Monitoring and Reporting (https://grants.nih.gov/grants/guide/url_redirect.php?id=82428).

A final RPPR, invention statement, and the expenditure data portion of the Federal Financial Report are required for closeout of an award, as described in the NIH Grants Policy Statement Section 8.6 Closeout (//grants.nih.gov/grants/guide/url_redirect.php?id=82420). NIH NOFOs outline intended research goals and objectives. Post award, NIH will review and measure performance based on the details and outcomes that are shared within the RPPR, as described at 2 CFR Part 200.301.

## Section VII. Agency Contacts

We encourage inquiries concerning this funding opportunity and welcome the opportunity to answer questions from potential applicants.

### Application Submission Contacts

eRA Service Desk (Questions regarding ASSIST, eRA Commons, application errors and warnings, documenting system problems that threaten submission by the due date, and post-submission issues)

Finding Help Online: https://www.era.nih.gov/need-help (https://www.era.nih.gov/need-help) (preferred method of contact)
Telephone: 301-402-7469 or 866-504-9552 (Toll Free)

General Grants Information (Questions regarding application instructions, application processes, and NIH grant resources)
Email: GrantsInfo@nih.gov (mailto:GrantsInfo@nih.gov) (preferred method of contact)
Telephone: 301-480-7075

Grants.gov Customer Support (Questions regarding Grants.gov registration and Workspace)
Contact Center Telephone: 800-518-4726
Email: support@grants.gov (mailto:support@grants.gov)

### Scientific/Research Contact(s)

Henry Burch, M.D.
National Institute of Diabetes and Digestive and Kidney Diseases
Telephone: 301-827-0827
Email: henry.burch@nih.gov (mailto:henry.burch@nih.gov)

### Peer Review Contact(s)

Examine your eRA Commons account for review assignment and contact information (information appears two weeks after the submission due date).

### Financial/Grants Management Contact(s)

Natasha Kimes, MBA
National Institute of Diabetes and Digestive and Kidney Diseases (NIDDK)
Phone-301-594-8853
Email: natasha.loveless@nih.gov (mailto:natasha.loveless@nih.gov)

# Section VIII. Other Information

Recently issued trans-NIH policy notices (//grants.nih.gov/grants/guide/url_redirect.php?id=11163) may affect your application submission. A full list of policy notices published by NIH is provided in the NIH Guide for Grants and Contracts (//grants.nih.gov/grants/guide/url_redirect.php?id=11164). All awards are subject to the terms and conditions, cost principles, and other considerations described in the NIH Grants Policy Statement (//grants.nih.gov/grants/guide/url_redirect.php?id=11120).

### Authority and Regulations

Awards are made under the authorization of Sections 301 and 405 of the Public Health Service Act as amended (42 USC 241 and 284) and under Federal Regulations 42 CFR Part 52 and 2 CFR Part 200.

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Weekly TOC for this Announcement (/grants/guide/WeeklyIndex.cfm?09-19-25)
NIH Funding Opportunities and Notices (/grants/guide/index.html)

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NIH National Institutes of Health (/grants/oer.htm)
Office of Extramural Research

(https://www.hhs.gov/) Department of Health and Human Services (HHS)

USA.gov (https://www.usa.gov/)

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